



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه احمد محمد يوسف
المحافظة : الجيزة
الحى / المدينة : اطيح
العنوان : 0
رقم البطاقة : 28402042104007 رقم التليفون : 01007809571
العمر (تاريخ الميلاد) : 04-02-1984 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمه احمد محمد يوسف National ID: 28402042104007	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٨٣

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمة
احمد محمد يوسف
كفر قنديل
مركز اقليم - الجيزة

٢٨٤٠٢٠٤٢١٠٤٠٠٧
١٩٨١/٠٩/١٤

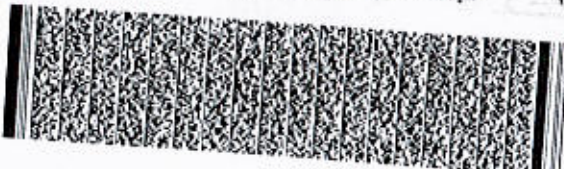
KD9487955

٢٨٤٠٢٠٤٢١٠٤٠٠٧
٢٠٢٣/٠٩/١٤

حاصله على دبلوم تجارة

أنشئ
مصلحة
متزوجة
حسن محمود علي محمود

البطاقة سارية حتى ٢٠٣٠/٠٩/١٤



Name : Fatima Ahmed Mohammed Youssef

Date : 02-07-2025 09:30

Branch : Maadi

Medical No. : 141353

Ref. Physician : Prof : Hrmal Hospital

Ins. Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 41-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

The previous scan images are not available for comparison

- Diffuse enhanced tracer uptake is seen at LV4.
- No other abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the rest of the spine and big joints.

Impression:

- No convincing evidence of metastatic bone disease.
- Equivocal tracer uptake at LV4, likely degenerative, for regional radiological correlation and follow up.

Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine

Name:	فاطمة احمد محمد يوسف	
MR. :	1698	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	
		إيماء للطلب المقدم من
	رقم طبي:	
		بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient has been on adjuvant tamoxifen since February 2025. Her HER2-directed therapy (Herceptin) was discontinued after three months. A contrast-enhanced CT of the chest, abdomen, and pelvis performed on 6 December 2025 demonstrated a stationary hepatic focal lesion (HFL) in segment 8 measuring 10 × 9 mm. After consultation with Dr. Mohamed Ismail, the plan was to continue tamoxifen and follow the hepatic lesion. The current medication regimen includes tamoxifen 20 mg daily, calcium 1 tablet × 1 × 90, and a single dose of Zoladex 10.8 mg subcutaneously. A follow-up triphasic CT CAP performed on 1 April 2026 showed the hepatic focal lesion in segment VIII unchanged at 11 mm, confirming a stationary course. The imaging was ordered for three-month surveillance, and the therapeutic plan was shifted to Zoladex combined with an aromatase inhibitor because tamoxifen was unavailable. Adjuvant therapy was adjusted to anastrozole (Arimidex) with supportive care. The patient received a dental consultation and was scheduled to receive a free dose of Zoladex on 1 July 2026, with the next clinic visit planned for 6 September 2026. The patient's breast cancer is classified as stage II (right breast, CT2, no metastatic disease, IDC grade 8/8, PR 4/8, HER2 3+). She was recommended to continue Zoladex and an aromatase inhibitor, with a repeat CT in April 2026 showing a single segment 8 hepatic focal lesion measuring 11 mm. Additional recommendations included zoledronic acid (Zometa), anastrozole, and Zoladex, with a follow-up scheduled for 2 July. A repeat CT CAP performed in August 2026 demonstrated a regressive course: the previously noted pulmonary and hepatic focal lesions were no longer appreciable, although a small lytic lesion persisted in the left femoral head. The plan is to continue anastrozole for three more months with supportive therapy and to obtain a bone scan. Zoladex was prescribed on 1 October 2026.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
		الطبيب المعالج :
		إيه مدحت بدوي محمد
يعتمد ,		



Patient Name : الأستاذة / فاطمة أحمد محمد يوسف	Request No. : 13006349
Patient No. : 28402042104007	Request Date : 23/01/2025
Path No. : 306/25	Verification Date : 12/02/2025
Sex : Female	Financial Category : نفقة التولية - اوراق دار السلام
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

- Perineural invasion: Not detected.
- Microcalcifications: Not detected.
- IPT:
- P63: Positive in all myoepithelial cells around DCIS lesion.
- SMA: Positive in all myoepithelial cells around DCIS lesion.

Diagnosis:

Right breast, simple mastectomy and SLNs Biopsy:

- Residual high grade DCIS (comedo and solid pattern), with positive lymphovascular invasion.
- Free deep surgical resection margin.
- Free skin.
- Free lymph nodes (0/6).
- Complete pathologic response to therapy.
- Stage: ypTis, ypN0.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 12/02/2025 10:51:43

Page 2 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mistr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / فاطمة احمد محمد يوسف	Request No.	: 13006349
Patient No.	: 28402042104007	Request Date	: 23/01/2025
Path No.	: 306/25	Verification Date	: 12/02/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right simple mastectomy and SLNs.

Gross Examination:

- Procedure: Right simple mastectomy specimen measured 20x17x4 cm with covering skin ellipse measured 16.5x7.5 cm.
 - Tumor extent: A rubbery greyish white tumor bed site.
 - Size: 7x5x2.5 cm.
 - Skin involvement: Nipple was unremarkable grossly.
 - Distance from margins: 2 cm away from deep margin.
 - Lymph nodes number and size: Fibrofatty tissue piece measured 2.5x2x0.5 cm, entangling 6 nodules (5 were frozen, one left for paraffin), up to 1x0.5 cm.
 - Frozen results: SLN: free in 0/5 lymph nodes.
 - Designation:
 - 1-7: Tumor bed site.
 - 8: Deep margin.
 - 9: Nipple.
 - 10-11: SLNs.
- Dissected by: Dr. Menan Hossam.

Microscopic Examination:

- Histological type: Residual high grade DCIS (comedo and solid)
- Angiolysis: Positive.
- Ductal carcinoma in situ: Residual high grade DCIS (comedo and solid pattern).
- Skin involvement: Free.
- Lymph node involvement: Free in (0/6) lymph nodes.
- Resection Margins: Free deep surgical resection margin.
- Histologic response to therapy: Complete pathological response.

Verified by :
Dr.Toka Medhat

Printing Date : 12/02/2025 10:51:43

Page 1 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة .. القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Msr El Qadima - Cairo, Egypt

Patient Name: Fatma Ahmed Mohamed Youssef	Date : 30/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 41 Year(s)
Medical ID: 2863/2024	Pat. ID: 76644

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Findings:

Known case of BC compared to last study dated 22/7/2024

I Chest:

- Right mastectomy with clear operative bed.
- No pathological right axillary lymph nodes.
- Still noted right lower lung lobe lateral basal segment fissural based 2 mm nodule.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

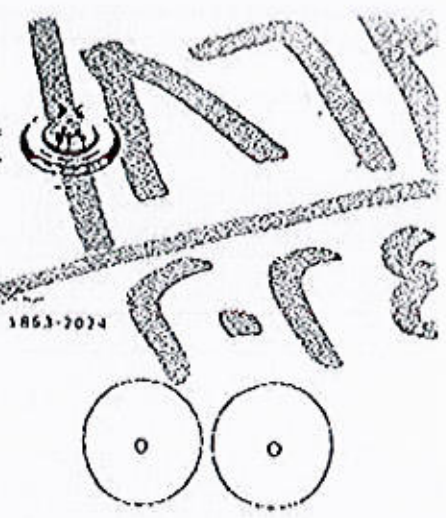
- Enlarged size of the liver with homogenous CT texture. Small segment VIII enhancing focal lesion 10 x9mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Scanned skeleton show L4 first degree lytic spondylololthesis,
- Small lytic lesion of left femoral head.
- **Opinion:**
- **Known case of BC compared to last study**
 - Progressive course regarding right hepatic lobe focal lesion .
 - Left femoral head lytic lesion for correlation with bone scan.
 - Rather stationary course of rest of study.

Dr/Reham Khedr...MD



Ministry of Health
Kingdom of Saudi Arabia

وزارة الصحة
المملكة العربية السعودية



MDT form

A-Patient ID:

1- Name: نائلة بنت يوسف

Sex:

3- Medical number:
Female

83212520
016 Med 1

1853-2024

2- ID number:

Patient Age: 41 Y

Family history

None

Phone

None

010/807571

CHI

None

REASON OF MDT

Follow up

Marital status

Married

NRS Score

None

GROUP

None

Date of MDT

2025-02-22

Comorbidity

None

Menopausal
status

None

Initial STAGE

None

Clinical data

Breast Imaging:

rt breast mass 32 x 14 mm at 5 o'clock, another lesion at 12 o'clock
2x1 cm, no pathological LH

Mets Workup:

ct cap 22/7/2024: only L4 lesion
bone scan 24/8/2024: equivocal L4 lesion
MRI lumbar spine: requested

DX
IOC

ER
8/8

PR
4/8

Her 2 Neu
3

SISH

KI67
15

T
2

N
0

M
0

Sonohistomimography

PATIENT HISTORY

triple +ve rt breast cancer

ICHP -> started 1st TC 4/9/2024

cesm: revealed multicentric process on rt breast & one clip is inserted

received NACT TC x6 cycle last 18/12/2024

HP v (-) -> clinically responding

POST Right simple mastectomy and SLNs

- Residual high grade DCIS (comedo and solid pattern), with positive lymphovascular invasion.
- Free deep surgical resection margin.
- Free skin.
- Free lymph nodes (0/6).
- Complete pathologic response to therapy.
- Stage: ypTis, ypN0.

MDT OUTCOME & PLAN

FOR TAM - HERCEPTIN 1 YR + Zoladex 10.8

SIGNATURE

MEDICAL ONCOLOGY

Signature

RADIOLOGY

Signature

Patient's Name: fatmaahmed mohamed yousef
Date: 2026-04-01
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Findings:

Known case of breast CA compared to last study dated 6/12/2025, the current study showed:

I Chest:

- Right mastectomy with clear operative bed.
- No pathological right axillary lymph nodes.
- Still noted two tiny right upper lobe pulmonary nodules.
- Still seen multiple bilateral scattered upper and lower lobe ground glass densities.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Enlarged size of the liver with homogenous CT texture showing no appreciable time interval changes regarding the appearance of the previously seen small segment VIII enhancing focal lesion still measuring 11 mm in diameter.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Scanned skeleton still shows first degree lytic anterolisthesis of L4 over L5 vertebra,
- Still noted small lytic lesion at the left femoral head.

Opinion:

- Stationary course since last study.

Dr.sherif Wagdy

Patient name: _____
MRN: _____

DOCTOR'S ORDERS

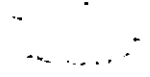
1A7 Y DOCTOR'S ORDERS
C.C.E. available

ER 8/8
PR 4/8

Diagnosis: Unit: 414 Diet Type:
hcg 12 + 3

Date	Time Of Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Execution
		→ Tamoxifen 20					
		Calcium					
		bone care					
		→ Herceptin 14.					
		→ C.Ca 1M IV.					
		→ B.S 7/2025: Equivocal Transect					
		uptake 1V4 likely degenerative					
		→ C.T 7/2028: H. F.I. → triphasic (B)					

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.
* Do NOT use abbreviations.
* This form is NOT intended for use for medications orders.



Patient Name	: الأستاذة / فاطمه احمد محمد يوسف	Request No.	: 13000063
Patient No.	: 28402042104007	Request Date	: 02/07/2024
Path No.	: 2203/24	Verification Date	: 14/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data

Right breast mass

Nature of specimen

Tru cut biopsy, collected at 1/7/2024, 10:30 am

Gross examination

Multiple(3) grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: Focal
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No

Diagnosis

Right breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II.

For further IHC (ER, PR, HER2, Ki67)



Signature

Dr. Rana Fathy
Dr.Heba Hassan

Verified by :

Dr.Heba Hasan





Patient Name	الأستاذة / فاطمه احمد محمد يوسف	Request No.	13000063
Patient No.	28402042104007	Request Date	02/07/2024
Path No.	2203/24	Verification Date	22/07/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	40 Y	Department	Out Patient
Referring Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Marwa Mahmoud.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady





Patient Name: Fatma Ahmed Mohamed Youssef	Date : 22/07/2024
Exam : C.T CHEST	Age : 40 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

- Scanned skeleton show L4 first degree lytic spondylololthesis, no worrisome lytic or sclerotic lesions or structural vertebral collapse.

Opinion:

- **Right pulmonary micronodule likely of insignificant.**
- **Mild hepatomegaly with no definite focal lesions on noncontrast .**

Dr. Mostafa Abdelwahab, MSc
Signature Prof. Menna Tallah Hanafy

29/07/2024 02:41 م



Patient Name: Fatma Ahmed Mohamed Youssef	Date : 22/07/2024
Exam : C.T CHEST	Age : 40 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Findings:

I Chest:

- Right breast UIQ heterogenous parenchyma is seen.
- No pathological right axillary lymph nodes.
- Right lower lung lobe lateral basal segment fissural based 2 mm nodule.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Enlarged size of the liver with homogenous CT texture. No focal lesions or contour irregularities on non contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Patient Name	: الأستاذة / فاطمة احمد محمد يوسف	Request No.	: 13000063
Patient No.	: 28402042104007	Request Date	: 02/07/2024
Path No.	: 2203/24	Verification Date	: 14/07/2024
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data

Right breast mass

Nature of specimen

Tru cut biopsy, collected at 1/7/2024, 10:30 am

Gross examination

multiple(3) grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic examination

* Histological type: Invasive duct carcinoma.

* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II

* DCIS: Focal

* Angiolymphatic: Couldn't be detected in the received biopsy

* Perineural invasion: No

Diagnosis

Right breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II.

For further IHC (ER, PR, HER2, Ki67)

Signature

Dr. Rana Fathy

Dr.Heba Hassan

Verified by :

Dr.Heba Hasan

Printing Date : 22/07/2024 10:53:22

Page 1 of 2

Printing By : marwa mohamed youssef



Patient Name: Fatma Ahmed Mohamed Youssef	Date : 29/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 40 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

- Right Probably benign breast mass mostly a fibroadenoma (BIRADS 3) for better histopathological assessment.
- Normal sonomammography of left breast (BIRADS 1)

Signature Dr. Mohamed Mohsen

29/07/2024 12:47 PM

Patient Name الأستاذ / فاطمه احمد محمد يوسف
 Patient No. 28402042104007
 Birth No. 2203/24
 Sex Female
 Age 40 Y
 Referring Doc.

Request No. 13000063
 Request Date 02/07/2024
 Verification Date 22/07/2024
 Financial Category اورام دار السلام - نفقة الدولة
 Department Out Patient
 Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

CO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1-10%; 2, between 11% and 33%; 3, between 34% and 66%; 4, between 67% and 80%; 5, between 81% and 100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
 Dr. Marwa Mahmoud.
 Dr. Sagda Elkady.

Verified by :
 Dr. Sagda El-Kady

Printing Date : 22/07/2024 10:53:06

Page 2 of 2

Printed By : .marwa mohamed youss





Echo report

Name: فاطمة - احمد

Date: 24/11/2024

٢٨٦٢

٢٤

EF	65%	55-75%	PWT	1.3	0.6-1.1cm
LVEDD	5.6	3.5-5.8cm	AO.R	3.7	2-3.7cm
LVESD	2.6	2-4cm	LA	3.9	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT		0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions (Normal)

LV wall thickness (Normal)

LV diastolic function (Normal)

SWMA (Normal)

LA and Aortic root dimensions (Normal)

Mitral Valve (Normal)

Aortic valve (Normal)

Tricuspid valve (Normal)

Pulmonary valve (Normal)

Right side and systolic function (Normal)

Intracardiac masses and effusion (Normal)

Conclusion: Normal Echo cardigrapy stady

Msc cardiology

د. البشير



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : فاطمة احمد محمد يوسف

Sex:

3- Medical number:

Female

83212520

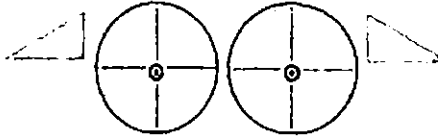
Old Med :

Old Med :

1863-2024

2-ID number:

Patient Age : 41 Y



Family history

Family history

-ve

Phone

Phone

0107809571

BMI

BMI

REASON OF MDT

REASON OF MDT

POST NACT

marital status

Marital Status

MARRIED

NRS Score

NRS Score

GROUP

GROUP

A

Date of MDT

Date Of MDT

18/09/2024 ☐

Co morbidity

Co Morbidity

Menopausal

status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

rt breast mass 32 x14 mm at 5 o'clock , another lesion at 12 o'clock
2x1 cm , no pathological LN

Mets Workup :

ct cap 22/7/2024: only L4 lesion
bone scan 24/8/2024: equivocal L4 lesion
MRI lumbar spine:requested

BX

IDC

BX COMMENT:

ER

8/8

ER COMMENT:

PR

4/8

PR COMMENT:

Her 2 Neu

3

Her 2 Neu COMMENT:

SISH

SISH COMMENT :

Ki67

15

T

2

N

0

M

0

Sonomammography

PATIENT HISTORY

triple +ve rt breast cancer

TCHP-->started 1st TC 4/9/2024

cesm: revealed multicentric process un rt breast & one clip is inserted

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

CONTINUE TREATMENT → TCHP x 6
BY REVIEWING CT NO SUSPICIOUS LESIONS

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شيماء لاشين	RADIOLOGY	RADIOLOGY د/ احمد حمادي
SURGERY	SURGERY د/ ابراهيم الزويت	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

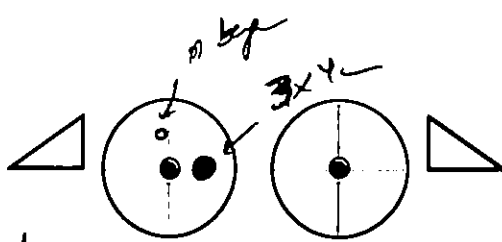
الاستراتيجية
صحة
مصر

MDT FORM

14/8/2024 G

Name	فاطمة محمد	MRN	1863/24
Age	45	ID	28402042104609
Menopausal status		Phone	010-7809571
NRS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity		REASON OF MDT	

Clinical Data

BX:	Endometrial carcinoma	R	L
ER:	8/8	PR:	4/8
HER 2:	Positive	Ki67:	Positive
Sonomammography:	Right breast lesion 32x14mm at 3 o'clock another lesion at 12 o'clock 2x1cm no features.		
Mets Workup:	free metastatic work up. CT chest - Right lower lung lobe Nodule 2mm (for follow up)	TNM c2.	

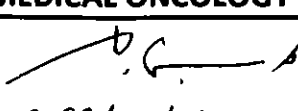
Recommendations:

1. GUS
2. Labs
3. CSRT the oc MDT
4. Bx

MDT OUTCOME & PLAN

Pt. will be for NMG (TCHP x 1) guided by clinical response
Surgical debr after CSRT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
 10/8/24					



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

المنظمة المصرية
طبعة
مصر

MDT FORM

/ / 20 ^{1174G}

Name	لطيفة محمد	MRN	1174G
Age	40 yrs	ID	1174G
Menopausal status	Pre-Menopausal	Phone	010-711-9071
NRS SCORE		Marital status	married.
BMI		Family history	-ve
CO morbidity	-ve	REASON OF MDT	1101097971

Clinical Data

BX: IDC, G-II

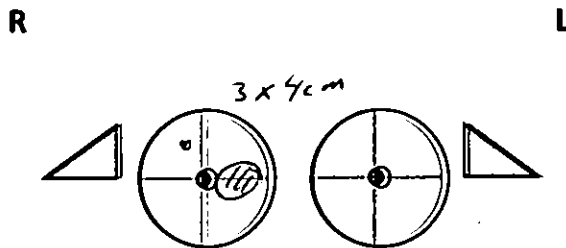
ER: 8/8 PR: 4/8 HER2: +3 Ki67: 15%.

Sonomammography:

* CESM (24/8/2024): Multi-centric Rt. Breast process.

Mets Workup:

* CT CAPE (22/7/2024): - only L4 lesion.
* Bone scan (24/8/2024): - Equivocal L4 lesion.
* MRI Lumbar spine → requested



TNM

Recommendations:

Triple Positive Rt. Breast Ca.
* Planned to start Neo-adj. TCHP. → started
1st TC on 4/9/2024.
CT2 cN1 M? → L4 lesion seen in both CT CAP, Bone scan. "Pending MRI".
↳ Indeterminate 3mm IN.

- * CESM → Revealed multi-centric Process in Rt. Breast. & one clip is inserted
- * There is single L4 lesion seen in CT CAP, Bone scan.

MDT OUTCOME & PLAN

* Kindly, advise for further plan of H. & MRI lumbar spine pending.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatry

الدور الثاني



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : ناطمة احمد محمد يوسف

Sex:

3- Medical number:

Female

83212520

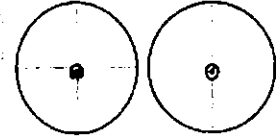
Old Med :

Old Med
1863-2024

2-ID number:

Patient Age : 41 Y

Post-NAC
clinical
CR



Family history

Family History
-ve

Phone

Phone
0107809571

BMI

BMI

REASON OF MDT

REASON OF MDT
POST NACT

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP
A

Date of MDT

Date of MDT

01/01/2025 ☐

Co morbidity

Co Morbid ty

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

rt breast mass 32 x14 mm at 5 oclock , another lesion at 12 oclock
2x1 cm , no pathological LN

Mets Workup :

ct cap 22/7/2024: only L4 lesion
bone scan 24/8/2024: equivocal L4 lesion
MRI lumbar spine: requested

Fix IDC	ER 8/8	PR 4/8	Her 2 Neu 3	SISH	KI67 15
Bx COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T 2	N 0	M 0
--------	--------	--------

Sonomammography

PATIENT HISTORY

triple +ve rt breast cancer
TCHP-->started 1st TC 4/9/2024
cesm: revealed multicentric process un rt breast & one clip is inserted
rceivd NACT TC x6 cycle last 18/12/2024
HP x 3 -->clinically responding

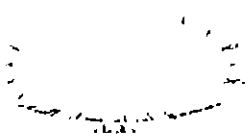
MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

in view of extensive area of micro calcification -->consider rt mastectomy & SLNB
(+/- reconstruction--->pt is refusing)

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شيماء لاشين	RADIOLOGY	RADIOLOGY د/ ناهد يحيى
SURGERY	SURGERY د/ ايمان شوقي	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Presidential Program for Quality Health
Ministry of Health and Population

البرنامج الرئاسي للصحة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : فاطمة احمد محمد يوسف

Sex:

3- Medical number:

Female

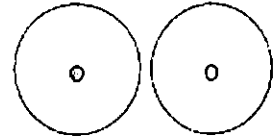
83212520

Old Med :

1863-2024

2-ID number:

Patient Age : 41 Y



Family history

Family history
+ve

Phone

Phone

0107809571

BMI

PMI

REASON OF MDT

Reason of MDT
L4 lesion

marital status

Marital status
+ve

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT
2025-02-22

Co morbidity

Co morbidity

Menopausal
status

Menopausal
status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

rt breast mass 32 x14 mm at 5 oclock , another lesion at 12 oclock
2x1 cm , no pathological LN

Mets Workup :

ct cap 22/7/2024: only L4 lesion
bone scan 24/8/2024: equivocal L4 lesion
MRI lumbar spine:requested

BX IDC	ER 8/8	PR 4/8	Her 2 Neu 3	SISH	KI67 15
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T 2	N 0	M 0
--------	--------	--------

Sonomammography

PATIENT HISTORY

triple +ve rt breast cancer
TCHP-->started 1st TC 4/9/2024
cesm: revealed multicentric process un rt breast & one clip is inserted
received NACT TC x6 cycle last 18/12/2024
HP x 3 -->clinically responding
POST Right simple mastectomy and SLNs
- Residual high grade DCIS (comedo and solid pattern), with positive lymphovascular invasion.
- Free deep surgical resection margin.
- Free skin.
- Free lymph nodes (0/6).
- Complete pathologic response to therapy.
- Stage: ypTis, ypN0.

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR TAM - HERCEPTIN 1 YR

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY فاطمة احمد محمد يوسف	RADIOLOGY	RADIOLOGY فاطمة احمد محمد يوسف
------------------	--	-----------	-----------------------------------

Patient Name : الأستاذة / فاطمة احمد محمد يوسف	Request No. : 13006349
Patient No. : 28402042104007	Request Date : 23/01/2025
Path No. : 306/25	Verification Date : 12/02/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right simple mastectomy and SLNs.

Gross Examination:

- * Procedure: Right simple mastectomy specimen measured 20x17x4 cm with covering skin ellipse measured 16.5x7.5 cm.
 - * Tumor extent: A rubbery greyish white tumor bed site.
 - * Size: 7x5x2.5 cm.
 - * Skin involvement: Nipple was unremarkable grossly.
 - * Distance from margins: 2 cm away from deep margin.
 - * Lymph nodes number and size: Fibrofatty tissue piece measured 2.5x2x0.5 cm, entangling 6 nodules (5 were frozen, one left for paraffin), up to 1x0.5 cm.
 - * Frozen results: SLN: free in 0/5 lymph nodes.
 - * Designation:
 - 1-7: Tumor bed site.
 - 8: Deep margin.
 - 9: Nipple.
 - 10-11: SLNs.
- Dissected by: Dr. Menan Hossam.

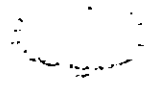
Microscopic Examination:

- * Histological type: Residual high grade DCIS (comedo and solid)
- * Angioinvasion: Positive.
- * Ductal carcinoma in situ: Residual high grade DCIS (comedo and solid pattern).
- * Skin involvement: Free.
- * Lymph node involvement: Free in (0/6) lymph nodes.
- * Resection Margins: Free deep surgical resection margin.
- * Histologic response to therapy: Complete pathological response.

Verified by :

Dr.Toka Medhat





Patient Name : الأستاذة / فاطمه احمد محمد يوسف	Request No. : 13006349
Patient No. : 28402042104007	Request Date : 23/01/2025
Path No. : 306/25	Verification Date : 12/02/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

* Perineural invasion: Not detected.

* Microcalcifications: Not detected.

IPT:

- P63: Positive in all myoepithelial cells around DCIS lesion.

- SMA: Positive in all myoepithelial cells around DCIS lesion.

Diagnosis:

Right breast, simple mastectomy and SLNs Biopsy:

- Residual high grade DCIS (comedo and solid pattern), with positive lymphovascular invasion.

- Free deep surgical resection margin.

- Free skin.

- Free lymph nodes (0/6).

- Complete pathologic response to therapy.

- Stage: ypTis, ypN0.

Signature

Dr. Walaa Ghonaim

Dr. Yasmine Essam

Verified by :

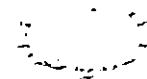
Dr. Toka Medhat

Printing Date : 12/02/2025 10:51:43

Page 2 of 2

Printing By : .marwa mohamed youss





Patient Name	: الأستاذة / فاطمة احمد محمد يوسف	Request No.	: 13006349
Patient No.	: 28402042104007	Request Date	: 23/01/2025
Path No.	: 306/25	Verification Date	: 12/02/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 40 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

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Right breast, simple mastectomy and SLNs Biopsy:

- Residual high grade DCIS (comedo and solid pattern), with positive lymphovascular invasion.

- Free deep surgical resection margin.

- Free skin.

- Free lymph nodes (0/6).

- Complete pathologic response to therapy.

- Stage: ypTis, ypN0.

Signature

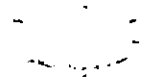
Dr. Walaa Ghonaim

Dr. Yasmine Essam

Verified by :

Dr.Toka Medhat





Patient Name	: الأستاذة / فاطمة احمد محمد يوسف	Request No.	: 13006349
Patient No.	: 28402042104007	Request Date	: 23/01/2025
Path No.	: 306/25	Verification Date	: 12/02/2025
Sex	: Female	Financial Category	: اوراء دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right simple mastectomy and SLNs.

Gross Examination:

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 - * Frozen results: SLN: free in 0/5 lymph nodes.
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 - 1-7: Tumor bed site.
 - 8: Deep margin.
 - 9: Nipple.
 - 10-11: SLNs.
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Microscopic Examination:

- * Histological type: Residual high grade DCIS (comedo and solid)
- * Angioinvasion: Positive.
- * Ductal carcinoma in situ: Residual high grade DCIS (comedo and solid pattern).
- * Skin involvement: Free.
- * Lymph node involvement: Free in (0/6) lymph nodes.
- * Resection Margins: Free deep surgical resection margin.
- * Histologic response to therapy: Complete pathological response.

Verified by :

Dr.Toka Medhat



PATIENT NAME: FATMA AHMED MOHAMMED YOSEF**GENDER:** Female**DATE :** 24-Aug-2024 10:56 AM**PATIENT ID:** 10124247965**AGE:** 40 y**ORDER:** 3002494,3002779**CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY****INDICATION:**

- Right breast pathologically proven malignancy (outside Baheya)

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV non-ionic contrast medium infusion 100 ml of omnipaque

REPORT:**RIGHT BREAST:**

- Contrast Enhanced Spectral Mammography LIQ and UIQ heterogenous malignant mass and non mass enhancement seen collectively measuring about 10x4 cm (related to the area of asymmetrical density harboring malignant pleomorphic microcalcifications seen in 2D mammography) corresponding to U/S to few intercommunicating ill defined spiculated mass lesions seen from 2 to 5 o'clock surrounded by distorted parenchyma and showing anterior ductal extensions reaching the nipple (associated with mild deviated nipple).
- Another UOQ well defined heterogeneously enhancing mass lesion measuring about 2x1.8 cm corresponding by U/S to 11 o'clock well defined microlobulated solid mass lesion with mild internal cystic changes measuring about 1.8x1 cm.
- Normal skin thickness and contour.
- Indeterminate axillary (level I) lymph nodes with mild increase of cortical thickness reaching about 3.4 mm.
- No pathological infra or supraclavicular lymph nodes.

LEFT BREAST:

- Contrast Enhanced Spectral Mammography showed moderate symmetrical back ground parenchymal enhancement denoting adenosis.
- No left breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Non specific axillary lymph nodes.

PATIENT: FATMA AHMED MOHAMMED YOSEF, 10124247965, Female, 40 y

Report version 1, 24-Aug-2024 12:22 PM

Page 1 of 2

www.Baheya.org

16602

مركز ب. هية
4 شارع طلحة بول شان الهدم بحمام دولي - الحيزةمستشفى ب. هية الشيخ زايد
أ. أ. الحيز الأول الشيخ زايد عام كلية الهندسة

OPINION:

- Right breast locally advanced pathologically proven malignant mass an non mass enhancing lesions with indeterminate axillary lymph nodes (BIRADS 6).
- Right UOQ suspicious heterogeneously enhanced mass lesion ??
- Multicentric process for histopathological correlation (BIRADS 4c).
- No left breast pathological mass or non mass enhancement (BIRADS 2)

Referral to clinic.

Reported By,

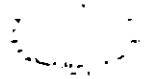
Dr. Asmaa Zeinhom,MSc

24-Aug-2024 12:22 PM

PATIENT: FATMA AHMED MOHAMMED YOSEF, 10124247965, Female, 40 y

Report version 1, 24-Aug-2024 12:22 PM

Page 2 of 2



Patient Name	: الأستاذة / فاطمة احمد محمد يوسف	Request No.	: 13000063
Patient No.	: 28402042104007	Request Date	: 02/07/2024
Path No.	: 2203/24	Verification Date	: 22/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 1/7/2024, 10:30 am.

Technique:

Sections were prepared from the paraffin block, processed in BenchMark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
PR: Moderate nuclear reaction in 5% of tumor cells (score 4/8).
Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
Ki67: 15% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
PT:
ER: Positive (score 8/8).
PR: Positive (score 4/8).
Her2: Positive (score 3+).
Ki67: Positive in 15% of tumor cells.

Scoring system: "ER, PR"

Verified by :

Dr. Sagda El-Kady

Printing Date : 22/07/2024 10:53:06

Page 1 of 2

Printed By : marwa mohamed youss





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

**MDT form**

A-Patient ID:

1- Name : ناطمة احمد محمد يوسف

Sex:

3- Medical number:

Female

83212520

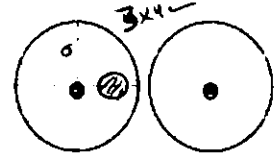
Cld Med :

Old Med :

1863-2024

2-ID number: 28402042104007

Patient Age : 41 Y

MDT

Family history

Family History

-ve

Phone

Phone

0107809571

BMI

BMI

32

REASON OF MDT

REASON OF MDT

No Cl

marital status

Marital Status

(MARRIED)

NRS Score

NRS Score

1

GROUP

GROUP

A

Date of MDT

Date Of MDT

14/08/2024 ☒

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data**Breast Imaging :**

rt breast mass 32 x14 mm at 5 oclock , another lesion at 12 oclock
2x1 cm , no pathological LN

Mets Workup :

free
ct chest: rt lower lung lobe nodules

BX IDC BX COMMENT:	ER 8/8 ER COMMENT:	PR 4/8 PR COMMENT:	Her 2 Neu 3 Her 2 Neu COMMENT:	SISH SISH COMMENT :	KI67 15
--	--	--	--	-------------------------------	--------------------------

T 2	N 0	M 0
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Sonomammography**PATIENT HISTORY**

ECHO / CESM pre MDT for signal dec

LABS**BONE SCAN****CLIP INSERTION****MDT OUTCOME & PLAN****MDT OUTCOME & PLAN**

PT. WILL BE FOR NACT TCHP X 6 CYCLES & CONSIDER RT WLE& SLNB

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شيماء لاشين	RADIOLOGY	RADIOLOGY د/ نور الهادي
SURGERY	SURGERY د/ ابراهيم عبدالرحمن	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition امراء حبيب



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مصر

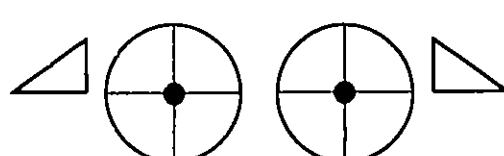
MDT FORM

/ / 20

G

Name	أحمد محمد	MRN	1172
Age	41	ID	2.22
Menopausal status		Phone	1251202-
NRS SCORE		Marital status	-100-78-9071
BMI		Family history	100-120-7729
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IDC			R L  T N M
ER:	PR:	HER 2:	KI67:	
8/8	4/8	3		
Sonomammography:				
Mets Workup:				

Recommendations:

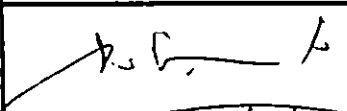
RT Breast Ca → CT2 - No. → need NACT TCX6ycl last
clinical response → for RTBCS + HPA3 18/12
+ SLN

MDT-OUTCOME & PLAN

In view of extensive area of microcalcifications → consider
RT. mastectomy & SLNRS (+/- resection)

pt. is referring.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاست المصيرية
طحة
مصر

18/10/2024

MDT FORM

/ / 20

G

Name	فاطمة محمد يوسف	MRN	1874/9-98
Age	40yr	ID	-1--78-9071
Menopausal status		Phone	-1--8907759
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	Postoperative

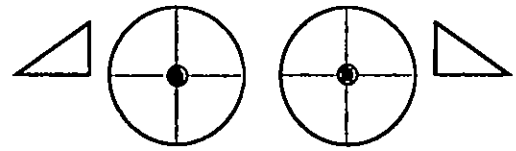
Clinical Data

BX: Post SM+SLNB
= Pathological Complete Response

ER: 818 PR: 418 HER 2: 3 Ki67: 15%

Sonomammography:

Mets Workup:



T N M

Recommendations:

Post SM+SLNB: Revealed Pathological
Complete Response, No surgical margins.
LN's (0/6) Stage T₀, No evidence of Neoadjuvant
ET.

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Echo report

Name:

Date: ٢/١/ 2025

Age:

١٨٦٢
٢٤

EF	65%	54-74%	PWT	3	0.5-1.1cm
LVEDD	44	3.5-5.7cm	AO	30	2.0-3.7cm
LVEDS	28	2.2-4.0cm	LA	33	1.9-4.0cm
FS	30	25-45%	RV	—	0.7-2.3cm (supine)
SWT	8	0.6-1.1cm	AO.CUSP	—	1.5-2.6cm

LV dimensions:

LV wall thickness:

LV diastolic function:

SWMA.

LA size

Aortic root dimension

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side size and systolic function

Intracardiac masses and thrombi

Pericardium

IVC diameter and collapsibility.

Conclusion:

Dr:

اسم المريض ثلاثي: فاطمة احمد
رقم الملف: 1876
٢٤



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امانة المراكز الطبية المتخصصة
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Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT: 84 Kg	HT: 162 cm	Blood Pressure	Blood Sugar
			BSA: 1.94			

☒ Treatment

☐ Follow-up

at Echo. → pending.

* Full lab, [18/7/2024]: OK.

Clinical findings:

* Bone scan [24/8/2024]:

Equivocal L4 lesion.

L4 also present in CT CAP.

* For: MRI lumbar spine to check nature of L4 lesion.

* CESM [24/8/2024]: Multicentric Rt Breast lesions.

Lg. Ben. M.D.T. after CESM.

* Clips (one Clip 1 is inserted).

	Doctor Order	الوقت
EF; %	* For:	
BP; 3wks	[C1] Taxotere 145mg → Please add 2A Dexam. + 2A Avil to the preparation, as the patient did not receive the steroid pre-medication.	
TEMP;	/Carbo. AUC(51	
HR:	- قرار Phesgo → Pending HRE	
HB:	lumbar spine [after L4 lesion is seen in bone scan].	
WBC:	- Controloc 40mg Tab. once daily for 20 days.	
PLT:	- Granitryl 2mg Tab. twice daily for 7 day.	
UREA:	- Methabioyer 8mg Tab. twice daily for 3 days.	
CREAT:	- Zoladex 10.8mg S.C. [For ovarian function Preservation]	
ALT:	د. جابر صروف PS أقبل	
AST:	PS + ديكسا	
T.BIL:	فريق المختبر	
D.BIL:	المرفقة لم تحضر أمس	

Doctor Signature: Date: 18/7/2024 Time: 9:12

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

Clinical findings:

✓ + no fever (illness)
ASG / —————
w/ p-wt loss & dehydration
(ISO) —————
C7C8 (T50m)
2019/2029
2004/10

	الوقت	فقر الدم انقراضی
EF; %	۱۵٪	
BP;	اعلیٰ سے کم ۷۸/۴۰	
TEMP;	امریات میں ۳۶-۳۷	
HR;	کھدائی میں ۲۰	
HB;	۱۰	
WBC:	۱۰	
PLT:	۱۰	
UREA:	مراہول میں ۱۵	
CREAT:	()	
ALT:	۹ / ۱۵۰	
AST:	۹ / ۱۵۰	
T.BIL:	تقریباً	
D.BIL:	۹ / ۱۵۰	

Doctor Signature: Date: 12 / 20 Time:

فاطمه احمد صحر
٢٤/١١٦٣

بعد لتأجيل

٣.١٤٥

AUC5

لمدة ١٢ أسابيع

أما في ١٢ / ١٠ / ٢٠٢٤
التي
بالتاريخ



C3 TXT / carbo
675g
١٠/١١

موجود في الجداول
١٦/١١/٢٠٢٤

عالمية
١٦/١١/٢٠٢٤

١٦/١١/٢٠٢٤
١٦/١١/٢٠٢٤
١٦/١١/٢٠٢٤

١ - P - ديكس + P - آسيل + P - إيميكس + P - لوسينول

١٠ - ١٢ - ١٤ - ١٦ - ١٨ - ٢٠ - ٢٢ - ٢٤ - ٢٦ - ٢٨ - ٣٠ - ٣٢ - ٣٤ - ٣٦ - ٣٨ - ٤٠ - ٤٢ - ٤٤ - ٤٦ - ٤٨ - ٥٠ - ٥٢ - ٥٤ - ٥٦ - ٥٨ - ٦٠ - ٦٢ - ٦٤ - ٦٦ - ٦٨ - ٧٠ - ٧٢ - ٧٤ - ٧٦ - ٧٨ - ٨٠ - ٨٢ - ٨٤ - ٨٦ - ٨٨ - ٩٠ - ٩٢ - ٩٤ - ٩٦ - ٩٨ - ١٠٠

٦١% (٤/٩/٢٠٢٤)
٥٤ (٦/١١/٢٠٢٤)
٥٠.٧ (last wt = 80kg)

١٤٥

AUC5



١٤ - ١٦ - ١٨ - ٢٠ - ٢٢ - ٢٤ - ٢٦ - ٢٨ - ٣٠ - ٣٢ - ٣٤ - ٣٦ - ٣٨ - ٤٠ - ٤٢ - ٤٤ - ٤٦ - ٤٨ - ٥٠ - ٥٢ - ٥٤ - ٥٦ - ٥٨ - ٦٠ - ٦٢ - ٦٤ - ٦٦ - ٦٨ - ٧٠ - ٧٢ - ٧٤ - ٧٦ - ٧٨ - ٨٠ - ٨٢ - ٨٤ - ٨٦ - ٨٨ - ٩٠ - ٩٢ - ٩٤ - ٩٦ - ٩٨ - ١٠٠

C1 LD
phego
(1200/600)
+
C4 TXT/
carbo
(AUC=5)
CLPHEGL
(6/11/2024)

اسم المريض ثلاثي: ...
رقم الملف: ...



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Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

PR

27/11/2024 Cells (5) TXT/Carbo
Cells (5) HP

امانة المراكز الطبية المتخصصة - جراحات
تاريخ: 27/11/2024
جراحات سرطان (5) AUC

الوقت	Doctor Order	الوقت
EF, %		1
BP;		1
TEMP;		1
HR;		1
HB;		1
WBC:	C2 phesgo	1
PLT:	+ C5 TXT/Carbo (AUC=5)	1
UREA:	CLPh/ 27/11/2024	1
CREAT:	CBC	1
ALT:	FXNS 27/11/2024	1
AST:	Seacr=1.1	1
T.BIL:		1
D.BIL:		1

Doctor Signature: Date: / / 20 Time:

$C_6TC + C_3phospho.$

2- کوئٹہ 149

unter 100 f. Aus der K-

دفاعت

6619 $\frac{1}{2}$ s. w. 1/4 sec 1nd -

— 1 —

Q — — — — — Answer

مسدود

As $\rightarrow$ $\vec{r}$ is in

50

2 1701 10

h. mot

6 TC (560)

3Pherg

D. P. [Signature]

lab 18/12

81.0 0.9

69
(۱) جنرل جاسٹس

Tax + Carbo
- 15/11

6Tc
C3 mesg.

۱۲/۱۸

Med New Coll

2nd 87 ml/ml
Carbu 560

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

Cypho:ga

MS1 5 600/600 22

الوقت

D.BIL:

New EF (2) 65% (27/11/2025)

Cyphesgox

CLPH / 2816

(8/1/2025)

حرف صوف
دائره
C. 50/1/1A

Page 12 of 64

suagey done pending result

CS
فحص جوف ٦٠٠/٦٠٠ نت المبد
باراسول ٥٠٠ م. رقم ٢ مات لسه ٤١٢٤
نوع ووقت فرجه مبد لسه ٤١٢٤
اول المبد ١٢٤٤

جاری
١٢٤٤

CS phago
CLPh / مبد
(29/1/2025)

صند مشوا
+ تسكن
١٢٤٤
١٢٤٩

MDT on 22-2-2025

TAM - Herup for ty

pr. did Rr size testing + SN
on 23/1/2025

4 pTis, 4 pNo 0/6

اسم المريض ثلاثي: 21266
رقم الملف: 1774/ع



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مستشفى أورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Su
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

1/3/2025

grade 1 PN.

Feb. 1/3/2025

TLC 5/3.8, Hb 12.1, plt 343

ALT 5, AST 12.2, s/c 0.9

Doctor Order

T. 0.34, C. 1.22

W 4.

O/E: - NAD

for C6 Hercept

EF; %

BP;

TEMP;

HR;

HB;

WBC:

PLT:

UREA:

CREAT:

ALT:

AST:

T.BIL:

D.BIL:

الوقت

Doctor Signature: Date: 1 / 20 Time:

22.3.2025

grade 1 pN.

EF on 22/3/2025
64%

O/E NAD

IC: Arthroly

C6 Hercept adjt.

for New Lab,
AIP, Cabin work

clinical pharmacy
of Herceptin
ریسورس اعلیٰ
C.C. 11/11/22

شہید ۶۰، کلمہ
ٹاموکسین ۱۰، ۲۰، ۳۰
کالسیا ۱۰۰
بوسری ۱۰۰
سورے ۱۰۰
میلو ۱۰۰
ارے ۱۰۰
سورے ۱۰۰
چائے ۲۰

و/دیا (۶۰)
۲/۱۱



.....إسم المريض ثلاثي:
.....رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Su.
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

.....
.....
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.....
.....
.....

	Doctor Order	الوقت
EF; %		
BP;	هرمستمر ١٥٥/٩٠ مم زئبق جلد لفتحه	
TEMP;		
HR;	تخطيط قاع موافق	
HB;	٦٥ ليدم (٩٠)	
WBC:	١٠٠٠٠	
PLT:	١٠٠٠٠	
UREA:	١٠٠٠	
CREAT:	١٠٠٠	
ALT:	١٠٠٠	
AST:	١٠٠٠	
T.BIL:	١٠٠٠	
D.BIL:	١٠٠٠	

Doctor Signature: Date: / / 20 Time:

حقیقی نام - ۱۳۶۵

دوبلہ سٹیشن

ناموسین

کانکریٹ

والدین

موروث

منازل

انسانی



C8 H(CS)

4/5

clph/2003

Echo 64% on 22/3

صرف حیرت

و شہو

0/8

for herreptin C9 600mg SQ

CO/0/8
0/CO
C9 Herceptin
S.C

Handwritten text in Urdu, possibly a signature or note.

5/5
625

دراختی - پروتھین

pt clo from itching 0/CO
for dermatology consult C.C.C

0/CO
0/CO

Handwritten text in Urdu, possibly a signature or note.

C9 Her.s.c
vapor/d.up
C.C.C 0/CO

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood :
			BSA:		/	

☐ Treatment

☐ Follow-up

ef 63.1

22/6/2025

Clinical findings:

Cia Herceptin

هيرسبتين - ٦ جم - تحت المراقبة

C1 X 1 X 5

C1 X 5 X 1

C1 X 1 X 1

C1 X 1 X 1

Doctor Order

C1 X 3 X 1

C1 X 1 X 1

C1 X 1 X 1

C1 X 1 X 1

تاموتكس ١٠ جم

كاربوبر

برابر اليف

هاتين ٣ جم

براستيغول

مستل تاكتو

مستل

كوتل لوت

١٠ جم

١٠ جم

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EF; %

BP;

TEMP;

HR;

HB;

WBC:

PLT:

UREA:

CREAT:

ALT:

AST:

T.BIL:

D.BIL:

Doctor Signature: Date: / / 20 Time:

PATIENT NAME: FATMA AHMED MOHAMMED YOSEF

GENDER: Female

DATE : 24-Aug-2024 03:32 PM

PATIENT ID: 10124247965

AGE: 40 y

ORDER: 3002504

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Warm tracer uptake is seen involving LV4 of equivocal nature ?? Degenerative ?? Metastatic for CT correlation and follow up.
Degenerative changes are seen over dorso-lumbar vertebrae.
Otherwise the rest of surveyed skeleton is free from serious bone lesions.
Both kidneys are normally visualized.

Conclusion:

Equivocal LV4 lesion for CT correlation and follow up to exclude the less likely possibility of metastatic osseous deposit.

Reported By,**Prof. Huda Fathy, Nuclear Medicine Consultant****25-Aug-2024 11:03 AM**

PATIENT NAME: FATMA AHMED MOHAMMED YOSEF

GENDER: Female

DATE : 24-Aug-2024 03:32 PM

PATIENT ID: 10124247965

AGE: 40 y

ORDER: 3002504

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

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Degenerative changes are seen over dorso-lumbar vertebrae.
Otherwise the rest of surveyed skeleton is free from serious bone lesions.
Both kidneys are normally visualized.

Conclusion:

Equivocal LV4 lesion for CT correlation and follow up to exclude the less likely possibility of metastatic osseous deposit.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

25-Aug-2024 11:03 AM

Patient name: FATMA AHMED MOHAMED

Date: Sunday, 9, June, 2024 (2 يونيو) 1445 3 ذو الحجة

Dear Prof Dr: ABD ELMONEM SHEHATA (The King of Surgery)

Duplex, sonomammography of the right breast revealed: -

An irregular, speculated, hypoechoic, nonparallel (taller than wide) mass with punctate calcification is seen at 2 O'clock, the lesion measures 3 x 3 x 2.5 cm.

Four, enlarged, lymph nodes are seen:

- One at 12 O'clock (1.6 x 1.5 x 0.7 cm).
- One at 9 O'clock (2.6 x 1.6 x 1 cm).
- Two, medial wall, axillary (1.4 x 1.1 x 0.8 cm).

CONCLUSION:-

- ⌘ A malignant-looking, right, breast mass (3 x 3 x 2.5 cm).
- ⌘ Lymph nodes enlargement, as described above.

Yours sincerely,

د: أحمد خيري شعراوي

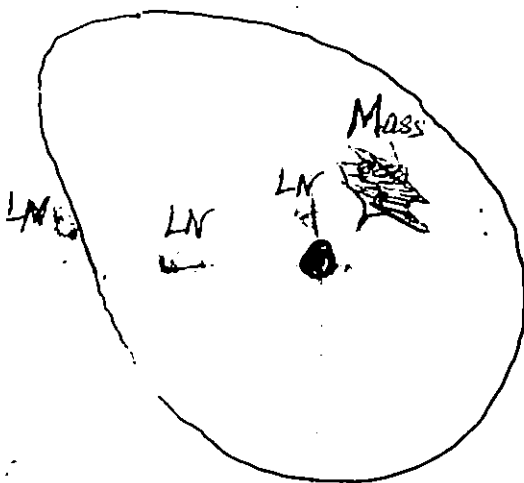
ماجستير (2006) الأشعة التشخيصية

جاء - القاهرة (1998)

أحمد خيري

استشاري أول الأشعة

Rt Breast



طبيب
أحمد خيري
1445

Patient Name: Fatma Ahmed Mohamed Youssef	Date : 22/12/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast cancer for follow up after NACT

Report:

Scattered fibroglandular tissue(ACR B)

Right Breast:

Still noted right UIQ coarse heterogenous microcalcifications.

With clip within measures 95 x 46 mm and 15 mm away from the nipple.

Stable right UOQ well defined oval shaped equal density lesion.

Complementary US shows **vanished** right 3 o'clock irregular hypodense mass lesion that was measuring about 32 x 14 mm.

Stable right 12 o'clock well defined isoechoic oval parallel mass measuring 20 x 11 mm.

Still noted multiple retroareolar anechoic dilated ducts the largest reaching 6 mm.

No pathological enlarged axillary lymph nodes

No pathological enlarged infra or supraclavicular lymph nodes.

Left Breast:

Scattered fibroglandular tissue(ACR B)

No suspicious micro calcifications or architectural distortion detected.

Complementary ultrasound no cystic or solid mass lesions.

Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.

Nonspecific axillary LN.

Opinion:

Status follow up showing good radiological response of right breast malignant mass (BIRADS 6).

Stable right Probably benign breast mass likely a fibroadenoma.

Signature Dr. Wafaa Ali

22/12/2024 2:20 PM



Patient Name: Fatma Ahmed Mohamed Youssef	Date : 29/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 40 Year(s)
Medical ID: 1863/2024	Pat ID: 76644

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Right breast palpable mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Right Breast:
 - Scattered fibroglandular tissue(ACR B)
 - Partially obscured high density lesion is seen in the right UIQ with adjacent spicules and overlying coarse heterogenous microcalcifications.
 - Right UOQ well defined oval shaped equal density lesion.
 - Complementary US shows right 3 o'clock irregular hypodense mass lesion measuring about 32 x 14 mm, 5mm from the skin, 37 mm from the nipple.
 - Right 12 o'clock well defined hypoechoic oval parallel mass measuring 20 x 11 mm.
 - Multiple retroareolar anechoic dilated ducts the largest reaching 6 mm.
 - No pathological enlarged axillary lymph nodes
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Left Breast:
 - Scattered fibroglandular tissue(ACR B)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound nc cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Right breast malignant mass (BIRADS 4c) for histopathological examination.



Patient Name: Fatma Ahmed Mohamed Youssef	Date : 29/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 40 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

- Right Probably benign breast mass mostly a fibroadenoma (BIRADS 3) for better histopathological assessment.
- Normal sonomammography of left breast (BIRADS 1)

Signature Dr. Mohamed Mohsen

29/07/2024 12:47 PM

Patient Name: Fatma Ahmed Mohamed Youssef	Date : 29/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 40 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Right Breast:
 - Scattered fibroglandular tissue(ACR B)
 - Partially obscured high density lesion is seen in the right UIQ with adjacent spicules and overlying coarse heterogenous microcalcifications.
 - Right UOQ well defined oval shaped equal density lesion.
 - Complementary US shows right 3 o'clock irregular hypodense mass lesion measuring about 32 x 14 mm, 5mm from the skin, 37 mm from the nipple.
 - Right 12 o'clock well defined hypoechoic oval parallel mass measuring 20 x 11 mm.
 - Multiple retroareolar anechoic dilated ducts the largest reaching 6 mm.
 - No pathological enlarged axillary lymph nodes
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Left Breast:
 - Scattered fibroglandular tissue(ACR B)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Right breast malignant mass (BIRADS 4c) for histopathological examination.



Patient Name: Fatma Ahmed Mohamed Youssef	Date : 22/07/2024
Exam : ultrasound ABDOMEN& PELVIS	Age . 40 Year(s)
Medical ID: 1863/2024	Pat. ID: 76644

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: bulky uterus ,endometrial thickness=1cm
cervical Nabothian cyst 8mm.

Opinion:

BULKY UTERUS

Signature Dr . Nermien Ramadan

PM 4:19 2024/07/22